A/PROF CHAMARA BASNAYAKE
MBBS, PhD, FRACP
Gastroenterologist & Endoscopist
PN 421926PK

FOCUS GASTROENTEROLOGY

13 May 2026

Dr Martyn Taylor
Waverley Family Healthcare

Dear Dr Taylor,

Re: Miss Isabella Muling
DOB: 22/08/2003 Phone: 0415650326
Address: 14 Eden Avenue, BOX HILL SOUTH, VIC, 3128
Email: bella.muling@gmail.com

Summary

- **Normal gastroscopy and colonoscopy with no evidence of inflammatory bowel disease or malignancy.**
- **Oesophageal biopsies negative for eosinophilic oesophagitis.**
- **Clarification of childhood surgical history of jejunal atresia.**
- **For intestinal ultrasound and trial of rifaximin for suspected bacterial overgrowth.**
- **No action required by you.**

I had the pleasure of reviewing Isabella following her gastroscopy and colonoscopy. You will be pleased to know that the procedures were structurally normal, with no evidence of Crohn's disease, ulcerative colitis, polyps, or malignancy.

The gastroscopy was performed primarily to investigate Isabella's dysphagia. The oesophagus appeared visually normal, and biopsies taken from two levels have excluded eosinophilic oesophagitis. Stomach biopsies showed a tiny amount of visual inflammation, but histology confirmed no significant pathology or *Helicobacter pylori* infection. Biopsies from the duodenum and terminal ileum were also unremarkable.

We spent some time clarifying Isabella's surgical history. While the referral mentioned duodenal atresia, Isabella clarified that she actually underwent surgery for jejunal atresia as an infant. This is clinically relevant as the majority of the jejunum cannot be reached via standard endoscopy. Given her history of intestinal surgery, she is at a higher risk for bacterial overgrowth and secondary motility issues, which likely contributes to her symptoms of bloating and constipation.

Impression and Plan I have opted to investigate the small bowel further using an intestinal ultrasound, which I have organised through St Vincent's. This will allow us to assess the four metres of small intestine not reached by the scopes for any evidence of thickening or dilatation.

I have also suggested a trial of rifaximin to "reset" the intestinal bacteria. I have explained to Isabella that this is a two-week course and, as it is not covered by the PBS, it will be an out-of-pocket expense. Regarding her constipation, I have advised her to use Osmolax daily to maintain a softer stool consistency. We discussed the importance of this in conjunction with her previous pelvic floor physiotherapy. I will review her in three months' time once the ultrasound is complete.

All appointments and enquires
Suite 201, 100 Victoria Parade
East Melbourne VIC 3002
Ph: 03 9650 7917
Fax: 03 9650 7910
Pager: 03 9387 1000
Email: office@focusgastro.com.au

Endoscopy locations
Epworth Freemasons Day Proc. Centre
320 Victoria Pde, East Melbourne, 3002
Hobson Healthcare Sydenham
566 Melton Hwy, Sydenham, 3037
St Vincent's Private Hospital
59 Victoria Pde, Fitzroy, 3065

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Yours sincerely,

A/Prof Chamara Basnayake
Gastroenterologist

Investigation Results:

MULING, ISABELLA MAE : SVP 926855

DOB: 22/08/2003 Age: 22 years

14 Eden Ave, Box Hill South 3128

Results from 24/04/2026 ..... Episode# 26P265149 Test Hist (Histology - Routine)

FINAL ANATOMICAL PATHOLOGY AAnat

Lab #: 2026B010798 **CLINICAL NOTES** Gas N . Mild gastric , dysphagia PHx duodenal atresia Duo N. Lower oesophagus N, mid oesophagus N. Random colon. **MACROSCOPIC DESCRIPTION** 1. Specimen labelled "Mid Oesophagus bx". Two mucosal fragments measuring 2-3 mm. A1. 2. Specimen labelled "Lower Oesophagus bx". Three mucosal fragments measuring 2-4 mm. A1. 3. Specimen labelled "Gastric bx". Two mucosal fragments measuring 2-3 mm. A1. 4. Specimen labelled "Duodenum bx". Five mucosal fragments measuring 2-5 mm. A1. 5. Specimen labelled "Random Colon bx". Four mucosal fragments measuring 1-5 mm. A1. **MICROSCOPIC DESCRIPTION** 1. Sections show oesophageal mucosa with a preserved architecture. No acute inflammation, ulceration or granuloma formation is seen. No eosinophilia is noted. No dysplasia is identified. 2. Sections show fragments of stratified squamous oesophageal mucosa. The architecture is preserved. No inflammation is seen. A fragment of intestinal mucosa is also included. This within normal morphological limits. 3. Sections show fragments of gastric mucosa including muscularis mucosa. No acute inflammation, ulceration or granuloma formation is seen. No intestinal metaplasia or dysplasia is present. No significant chronic inflammation is noted. Helicobacter pylori-like microorganisms are not identified on the H&E sections. 4. Sections show fragments of duodenal mucosa including muscularis mucosa and Brunner's gland. The villous architecture is preserved. No intra-epithelial lymphocytosis is seen. No acute inflammation, ulceration or granuloma formation is noted. No dysplasia is present. 5. Sections show fragments of large intestinal mucosa including muscularis mucosa. The crypt architecture is preserved. No acute inflammation, ulceration or granuloma formation is seen. No thickening of the subepithelial collagen plate is noted. No intra-epithelial lymphocytosis is present. No dysplasia is identified. **PATHOLOGIST'S OPINION 1. Mid oesophageal biopsy: Within normal morphological limits. 2. Lower oesophagus biopsy: Squamous mucosa, within normal morphological limits. 3. Gastric biopsy: Antral mucosa, within normal morphological limits. 4. Duodenal biopsy: Within normal morphological limits. 5. Random colon biopsy: Within normal morphological limits.** Pathologist: Dr Zakariya Kanaan

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